

COGNITIVE BEHAVIORAL THERAPY (CBT) – SESSION 2

New Case: Aryan – Homework Review & Formulation

Full Detailed – More Realistic Style

SESSION INFORMATION

Client:	Aryan (32 y/o Male)	Therapist:	Dr. A.
Session #:	2 of 18–20	Focus:	Homework review + 5-area formulation

PRE-SESSION THERAPIST NOTES

Client brought the diary but only completed entries on 4 out of 7 days (realistic). He showed some skepticism about whether therapy will help. Clear patterns visible in the diary: highest anxiety in evenings while preparing for next day and before meetings. Dominant thoughts center around fear of being seen as incompetent. Today's plan: Review homework honestly, develop 5-area formulation, identify key negative automatic thoughts, and assign new homework.

SESSION START & HOMEWORK REVIEW

Therapist: Good morning, Aryan. How has your week been? I see you brought the diary. How did it go with filling it?

Client: It was okay... I didn't do it every day though. I forgot on some days and on other days I just didn't feel like writing. I only have 4 entries.

Therapist: That's completely fine. Even 4 entries are helpful. Let's look at what you did write. Can you tell me about one of the days that stood out?

Client: On Tuesday evening while preparing for a code review, my mood was really low — like 3 out of 10. I kept thinking "If I miss something in the review, they'll think I don't know what I'm doing." I stayed up late checking everything again.

Therapist: Thank you for sharing that. So the situation was preparing for the code review, the thought was about being seen as incompetent, your mood dropped, and you ended up staying up late. Do you notice any pattern when you look at the entries?

Client: Yeah... mostly in the evenings when I'm preparing for the next day, and before meetings.

5-AREA FORMULATION

Therapist: Last session we talked about how thoughts, feelings, and behaviors are connected. Today let's expand that into the full 5-area model we often use in CBT. This helps us see the bigger picture more clearly.

Therapist draws the 5 areas on paper and fills it together with the client using Tuesday's situation:

Situation: Preparing for code review in the evening

Thoughts: "If I miss something, they'll think I don't know what I'm doing. They'll start doubting me."

Emotions: Anxiety (8/10), Shame (6/10)

Physical sensations: Racing heart, tight chest, sweaty palms

Behaviors: Over-checking code repeatedly, staying up very late, avoiding asking teammates for help

Therapist: When we look at all five areas together, we can see how they influence each other. Changing one area can help shift the whole cycle. Does this make sense to you?

Client: Yeah... I can see how my thoughts are making me stay up late, which then makes me more tired and anxious the next day.

IDENTIFYING KEY NEGATIVE AUTOMATIC THOUGHTS

Therapist: From the diary and what you've shared, a few thoughts seem to come up strongly and repeatedly. Let's note them down:

1. "If I make a mistake, they will think I don't know what I'm doing."

2. "I have to be perfect or they will start doubting me."

Therapist: Which of these feels strongest or most painful right now?

Client: The second one — "I have to be perfect or they will start doubting me." That one feels very heavy.

BEGINNING COGNITIVE WORK

Therapist: Let's gently look at this thought: "I have to be perfect or they will start doubting me." On a scale of 0–100, how much do you believe this right now?

Client: About 75%.

Therapist: Okay. What evidence do you have that supports this thought?

Client: I've seen seniors point out small mistakes in reviews. Once I made an error in a presentation and felt like they lost some confidence in me.

Therapist: Good. Now, what evidence might go against it — even small things?

Client: (thinking) Most of my work is actually fine. I've been appreciated for some features I built. Even when I made that mistake, the project still worked.

Therapist: That's helpful. If we were to create a more balanced thought, what could it be?

Client: Maybe: "I don't have to be perfect to be competent. Making small mistakes doesn't mean people will completely doubt me."

Therapist: That's a good start. How much do you believe this new thought right now?

Client: About 45%. It feels a bit better, but I still don't fully believe it.

HOMEWORK ASSIGNMENT

Therapist: For this week, I'd like you to continue the diary, and add one small task:

1. Continue the daily mood + thought diary (try to do it at least 5–6 days)
2. For the thought "I have to be perfect or they will start doubting me," write down at least 2 pieces of evidence for and 2 pieces against it
3. Try one small behavioral change: When preparing something, set a reasonable time limit and stop when the timer ends (even if it feels incomplete). Notice what happens.

Client: I'll try... but I might forget the diary again.

Therapist: That's okay. Just do what you can. We'll work with whatever you bring next time.

SESSION SUMMARY & FEEDBACK

Therapist: Today we reviewed your diary, developed a fuller 5-area formulation, identified your key thoughts, and started gently challenging one of them. How are you feeling about today's session?

Client: It was okay. The formulation helped me see things more clearly. I still don't know if all this will actually change anything though.

Therapist: That's a fair feeling to have. We'll keep working step by step and see what happens. Any questions?

Client: Not really.

POST-SESSION THERAPIST NOTES

- Client completed homework partially (4/7 days) — realistic.
- Some ongoing skepticism about therapy expressed.
- 5-area formulation developed collaboratively and well understood.
- Key negative automatic thought identified. Initial cognitive restructuring started with moderate belief shift (75% → 45%).
- Homework assigned with realistic client hesitation noted.
- Next session: Review new homework, continue cognitive restructuring, introduce Worry Time technique.

Disclaimer: This is a completely fictional but realistic simulation created solely for educational and AI training purposes. It does not represent any real person or therapy.